

Q What if I am too anxious and nervous about labor and delivery to do anything?

It's possible for your doctor to prescribe tranquilizers to help you endure labor and delivery, if you really are very anxious.

Tranquilizers make it easier for some pregnant women who are very anxious to more fully participate in the labor and delivery process. Each woman's reaction to these drugs is different. Some women find that they are able to cope more easily with labor and delivery with the help of the gentle drowsiness that tranquilizers afford. Other women feel like the drugs make them feel out of control during an extremely important moment in their lives, and too sleepy to participate fully in the events at hand. Tranquilizers don't relieve labor pain, but they can work in combination with your opioid analgesics to help you feel more comfortable than you would with the pain medications alone. Because tranquilizers can affect the breathing of your baby, they are given well before you will need to begin to push.

Q What is a walking epidural? Is it right for me?

Many women are intrigued by the idea of a walking epidural, because they'd like to walk around the hospital during labor with reduced pain. But it's common for women who have this procedure to stay in bed after it's been done. Walking epidurals aren't available at all hospitals, and they don't offer as much pain relief as a regular epidural. If you're interested in having this procedure done, check with your doctor before your delivery to see if the anesthesiologists at the hospital where you'll be delivering are trained to perform walking epidurals.

More than **60%**
of American women choose
to have an epidural for pain
relief during their labor.

Q What is an epidural, and how does it work?

An epidural is a procedure that injects painkilling drugs directly into the epidural space around your spinal cord through a fine tube. It works by numbing feeling below the waist, so is a very effective form of pain relief.

An anesthesiologist first gives you a local anesthetic injection to numb the section of your mid-to-low-back where the epidural goes in. He or she then inserts a long, hollow needle carrying a thin tube, or catheter, into the epidural space. (You may be asked to curl up on your side, or to sit up and bend forward for this part, since it opens the space between your vertebrae.) The anesthesiologist removes the needle, leaving the catheter in place. The painkiller (a mixture of anesthetic and narcotic) is either injected into your spine through the catheter, or pumped in intravenously, to block the nerve pathways that run from your uterus to your brain. Although epidurals traditionally caused complete numbness, many now allow sensation without pain, which means you can feel to push when the time comes.



Top offs The painkilling drugs can be topped off by injecting through the small catheter placed in the spine.

Q What should I expect if I want to ask for opioid analgesics?

Opioid analgesics are derived from the seeds of the opium poppy, which gives us morphine.

They offer limited relief from labor pain that can last for between two and four hours per dosage. They block the pain as it travels down your spinal column by "switching off" the cells in your pain receptors. The drug doesn't have any effect on your contractions; it dulls the sensation while letting your body go about its important work. Dosages are administered by your doctor usually via an injection in your thigh or buttocks, although they may also be given intravenously. (The pain relief will be quicker intravenously—becoming active within two or three minutes, compared with 20–30 minutes by injection.) Depending on how long your labor lasts, you can be given additional injections after the pain relief begins to wear off (within two to four hours). Some are likely to have more significant side effects on your baby's alertness, breathing, and feeding—it can take

several days after birth for the effects of the drug to wear off in a newborn, depending upon how soon after your last dose the baby was born. For this reason, doctors don't like to give the drug when it looks like your baby will be born during the next two or three hours. Depending on the drug and your reaction to it, you may feel nauseous or even vomit. Note that none of the analgesics will give you a pain-free labor, and for some women they simply don't work. These drugs can be useful if your first stage labor is lasting a long time and you need some sleep, since they can help you to relax.

Q Can I have opioid analgesics if I want a water birth?

Yes, although you will need to have had the drug in your system for at least two hours before you get into the pool. This is just to make sure that you don't feel overly drowsy in the water. Of course, the water itself will help with pain relief.